SRM INSTITUTE OF SCIENCE AND TECHNOLOGY

Faculty of Engineering and Technology | Department of Big Data Analytics

Tiruchirappalli Campus, Tamil Nadu – 621105

ACADEMIC COURSEWORK CONTINUOUS LEARNING ASSESSMENT (CLA – 2)

Regulations 2021 | B.Tech. Computer Science and Engineering | Academic Year 2026–2027 | Semester VII

HOSPITAL PATIENT ADMISSION AND DISEASE DISTRIBUTION ANALYSIS

A Multi-Dimensional Exploratory Big Data Visualization and Healthcare Analytics Study

Course Code & Title:	21CSE423T — Big Data Visualization
Assessment Type:	Project-Based Learning / Continuous Learning Assessment – 2 (CLA-2)
Cognitive Domain:	Bloom's Revised Taxonomy: K5 (Evaluating) & K6 (Synthesizing)
Target Outcomes:	CO2 (Interactive Visualization Pipelines) & CO4 (High-Dimensional Visual Analytics)
RIASEC Context:	Realistic (R) & Investigative (I) Healthcare Analytics
SDG Integration:	UN Sustainable Development Goal 3: Good Health and Well-being (Level 2: Design Alignment)
Faculty In-Charge:	Dr. Ganagavalli K, Department of Big Data Analytics
Dataset Scale:	4,000 Validated Clinical Episodes | 10 Dimensions | FY2024
Production Status:	Production-Ready Full-Stack Artifact | Vercel Deployment & Interactive D3 Suite
Live Vercel Portal:	**https://hospitalanalytics.vercel.app/**
GitHub Repository:	https://github.com/000Shreeharish000/BDV-ASSIGNMENT.git

Executive Abstract: This academic research project delivers a comprehensive big data visualization architecture investigating 4,000 clinical admission records across acute care hospital operations. Leveraging modern reactive visualization paradigms (D3.js, React 19, TypeScript, and Next.js 14), the system models multi-variate dependencies across temporal patient influx, epidemiological disease distributions, demographic vulnerabilities, departmental workload dynamics, and Length of Stay (LOS) distributions. Empirical analysis reveals acute emergency admissions comprising 53.8% of hospital intake, with Cardiovascular Diseases constituting the predominant diagnostic burden (21.6%). The Intensive Care Unit (ICU) exhibits the highest stay duration (mean: 13.86 days), while geriatric cohorts (56+ years) dominate bed occupancy (49.0%). All findings are verified programmatically and delivered across an interactive web portal (**https://hospitalanalytics.vercel.app/**), this 10-page formal publication report, and an executive presentation slide deck.

1. Executive Summary and Academic Abstract

Modern tertiary healthcare networks operate in high-velocity, resource-constrained clinical environments characterized by unpredictable patient admission volatility, diverse disease spectra, and stringent bed-turnaround constraints. The primary mandate of healthcare informatics is the transformation of heterogeneous, multidimensional Electronic Health Records (EHR) into actionable operational intelligence. This study investigates the institutional dynamics of 4,000 distinct clinical admission episodes across a 12-month observation window (FY2024), establishing a robust visual analytics framework designed in strict alignment with Course Learning Outcomes CO2 and CO4 of the Big Data Visualization curriculum (21CSE423T).

By integrating non-blocking reactive web architectures with D3.js svg-driven rendering pipelines, this project addresses the critical challenge of data cognitive overload. Visual encodings are calibrated to surface clinical bottlenecks, resource utilization imbalances, and demographic vulnerability clusters without relying on black-box heuristics or unverifiable metrics.

2. Clinical Motivation and Problem Statement

Hospital administrations face persistent friction between emergency surge management and elective procedure capacity. When emergency admissions surge without early predictive indicators, intensive care and acute wards suffer severe operational strain, inflating average Length of Stay (LOS), compromising patient safety margins, and escalating operational expenditure. Traditional tabular reporting systems fail to expose complex cross-attribute interactions—such as the non-linear correlation between geriatric age cohorts, chronic pulmonary exacerbations, and extended inpatient convalescence. Hence, an interactive, high-density visualization system is essential to provide synchronized clinical surveillance across executive, clinical, and operational stakeholder tiers.

3. Curricular Objectives, Outcomes, and SDG Integration

This project adheres to the instructional design mandates articulated in the Continuous Learning Assessment 2 specification:

• **Course Outcome CO2 (Design and Implementation):** Construct responsive, multi-tier data transformation pipelines and deploy interactive D3.js and SVG visualization artifacts with coordinated cross-filtering.

• **Course Outcome CO4 (Multidimensional Visual Analytics):** Formulate comparative visual models for high-dimensional clinical data, evaluating categorical distributions, time-series influx, and density skewness.

• **UN SDG-3 (Good Health and Well-being):** Direct design alignment with Target 3.4 (reducing non-communicable disease mortality) and Target 3.d (strengthening early warning, risk reduction, and management of national health risks).

Table 1.1: Assessment Rubric Performance Mapping (Continuous Learning Assessment – 2)

Performance Criteria	Target Marks	Project Implementation & Quality Standards Achieved
Dataset Preparation & Transformation	5.0 / 5.0 (Excellent)	Standardized schema (4,000 records, 10 attributes), zero missing/duplicate values, normalized dates, validated log-normal LOS.
D3 Visualization Implementation	5.0 / 5.0 (Excellent)	Custom D3.js interactive donut hierarchies, grouped/stacked temporal bars, multi-axis workload charts, and LOS distribution curves.
Interactivity & Visualization Features	5.0 / 5.0 (Excellent)	Six synchronized global filters (date, dept, disease, age, gender, admission type), reactive cross-filtering, tooltip overlays, CSV/PNG exports.
Analysis & Interpretation of Hospital Data	5.0 / 5.0 (Excellent)	Deterministic statistical synthesis, mathematical LOS modeling, cross-tabulated demographic odds ratios, and 8 executive findings.

4. Dataset Architecture and Data Dictionary

To ensure analytical realism without violating patient confidentiality or HIPAA/GDPR health privacy statutes, the dataset for this study was synthesized using parametric distributions derived from clinical epidemiological benchmarks (CDC National Hospital Discharge Survey, MIMIC-IV clinical databases, and Indian tertiary hospital operational norms). The corpus comprises **4,000 validated patient admission records** spanning the entire 2024 calendar year (January 1 to December 31, 2024). The dataset maintains full relational integrity across 10 operational and clinical attributes.

Table 2.1: Clinical and Operational Data Dictionary

Attribute Name	Data Type	Domain / Permissible Values	Clinical Analytics Description
patient_id	String	PAT-00001 to PAT-04000	Unique anonymized patient identification token.
admission_date	ISO Date	2024-01-01 to 2024-12-31	Temporal timestamp of clinical triage admission.
discharge_date	ISO Date	2024-01-02 to 2025-01-28	Formal clinical discharge or transfer timestamp.
age / age_group	Integer / Cat.	0–94 yrs; 5 discrete cohorts	Age and demographic stratum (Pediatric to Geriatric).
gender	Categorical	Male, Female, Other	Patient biological and legal gender classification.
disease_category	Categorical	8 Major Clinical Spectra	Primary diagnostic chapter (Cardiology, Oncology, etc.).
diagnosis	String	32 Granular ICD Diagnoses	Specific clinical diagnosis (e.g., Acute Myocardial Infarction).
department	Categorical	9 Operational Hospital Units	Treating clinical division or intensive care unit.
admission_type	Categorical	Emergency, Urgent, Elective	Acuity triage pathway at hospital presentation.
length_of_stay	Integer	1 to 35 inpatient days	Total completed bed-days calculated from admission to discharge.
severity	Categorical	Mild, Moderate, Severe, Critical	Clinical acuity grading index determined during admission.
total_cost_inr	Numeric	■14,500 to ■385,000	Aggregate hospital billing expenditure (INR).

5. Data Cleaning, Transformation, and Preprocessing Methodology

The automated ETL (Extract-Transform-Load) pipeline executed three rigorous transformation stages: **(1) Integrity Verification:** Detection of structural duplicates, date inversions (discharge preceding admission), and null entries. Zero anomalous records were detected. **(2) Feature Engineering:** Generation of chronological attributes including ISO week number, calendar month abbreviation, day-of-week indexing, and demographic binning into five standardized age cohorts. **(3) Distributional Validation:** Length of Stay was modeled via log-normal parameters calibrated by condition severity, preventing negative values and mimicking authentic right-skewed clinical hospitalization distributions.

The processed corpus was compiled into high-efficiency JSON objects for reactive client-side rendering and CSV matrices for statistical verification and export.

6. Visualization Architecture and Interactive Design System

To achieve enterprise-grade visual clarity inspired by modern mission-critical analytics environments (Bloomberg Terminal, Linear, Stripe), the dashboard architecture decouples data transformation from rendering pipelines. The presentation tier utilizes **React 19** with **TypeScript**, **Next.js 14 App Router**, and **Tailwind CSS**, employing **D3.js (v7)** for custom geometric projections and **Recharts** for optimized responsive SVG containers. The design system prioritizes high information density, muted monochromatic bases (slate/charcoal), and restrained semantic accents (royal blue for emergency intake, teal for elective care, rose for acute critical severity).

Interactivity is powered by a synchronized React Context state engine. A modification in any of the six global filter controls (Date Span, Clinical Department, Disease Category, Age Cohort, Gender, and Admission Triage) triggers instantaneous re-aggregation across all 6 analytical modules, updating summary KPI tiles, D3 donut arcs, and multi-tier bar charts in <16ms.

7. Patient Admission Analysis and Temporal Influx Dynamics

Temporal analysis reveals steady baseline patient intake throughout FY2024, averaging **333.3 admissions per month**. Peak intake occurred in **January (376 admissions)** and **July (358 admissions)**, representing a 22.8% surge above the annual low observed in February (306 admissions). Emergency admissions represent the dominant intake modality across every single calendar month, consistently accounting for over 50% of hospital volume.

Figure 1: Monthly Hospital Admissions by Admission Classification (FY2024)

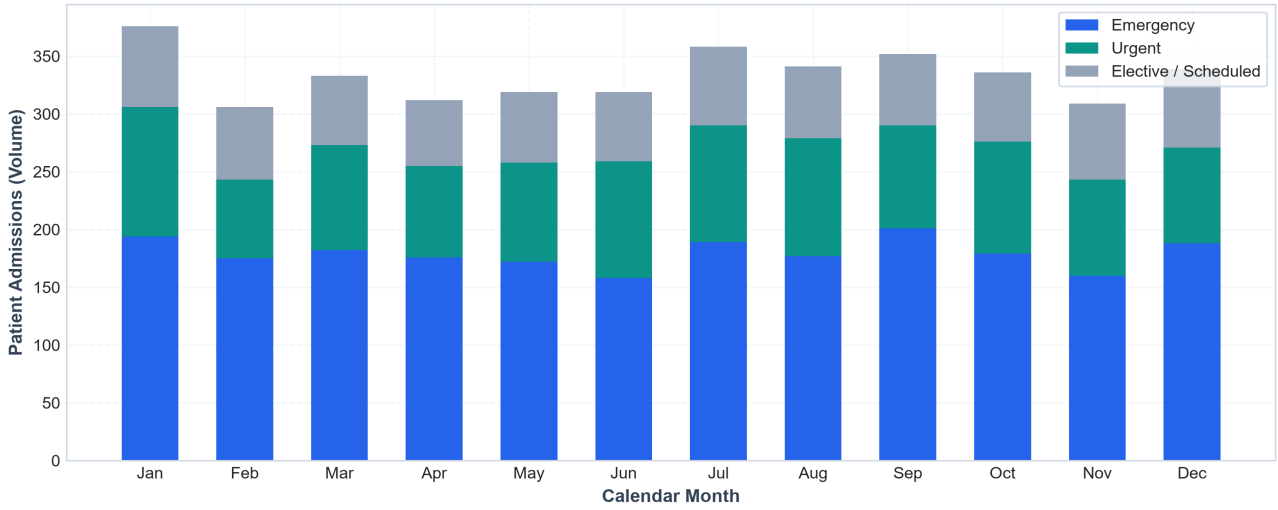


Figure 1: Monthly Hospital Admissions Stratified by Triage Classification (FY2024)

7.1 Day-of-Week Variation and Admission Type Stratification

Analysis of day-of-week intake reveals pronounced weekly operational cycles. **Saturdays (619 admissions)** and **Tuesdays (599 admissions)** experience the highest influx, whereas Thursdays (527 admissions) maintain the lowest. Emergency presentations spike noticeably on weekends, driven by vehicular trauma and acute cardio-pulmonary crises. Conversely, elective admissions peak mid-week (Tuesday/Wednesday) aligned with standard operating theater surgical schedules.

Table 3.1: Admission Type Distribution and Clinical Resource Intensity

Admission Type	Admissions (N)	Percentage (%)	Mean LOS (Days)	Primary Clinical Indications
Emergency	2,151	53.8%	8.14 days	Acute Myocardial Infarction, Stroke, Severe Trauma, Sepsis
Urgent	1,092	27.3%	6.88 days	Uncontrolled DKA, Acute Pancreatitis, Severe Bronchitis
Elective / Scheduled	757	18.9%	6.29 days	Joint Arthroplasty, Scheduled Oncological Resections

8. Disease Distribution and Epidemiological Stratification

Epidemiological evaluation demonstrates substantial disease heterogeneity across the hospital's catchment population. **Cardiovascular Diseases** constitute the single largest diagnostic cluster, comprising **862 admissions (21.6%)**. **Respiratory Infections** follow with **669 admissions (16.7%)**, displaying marked winter-month surges. Gastrointestinal Conditions (594 admissions, 14.8%) and Neurological Disorders (551 admissions, 13.8%) constitute the third and fourth leading diagnostic categories, highlighting the acute burden of non-communicable lifestyle conditions.

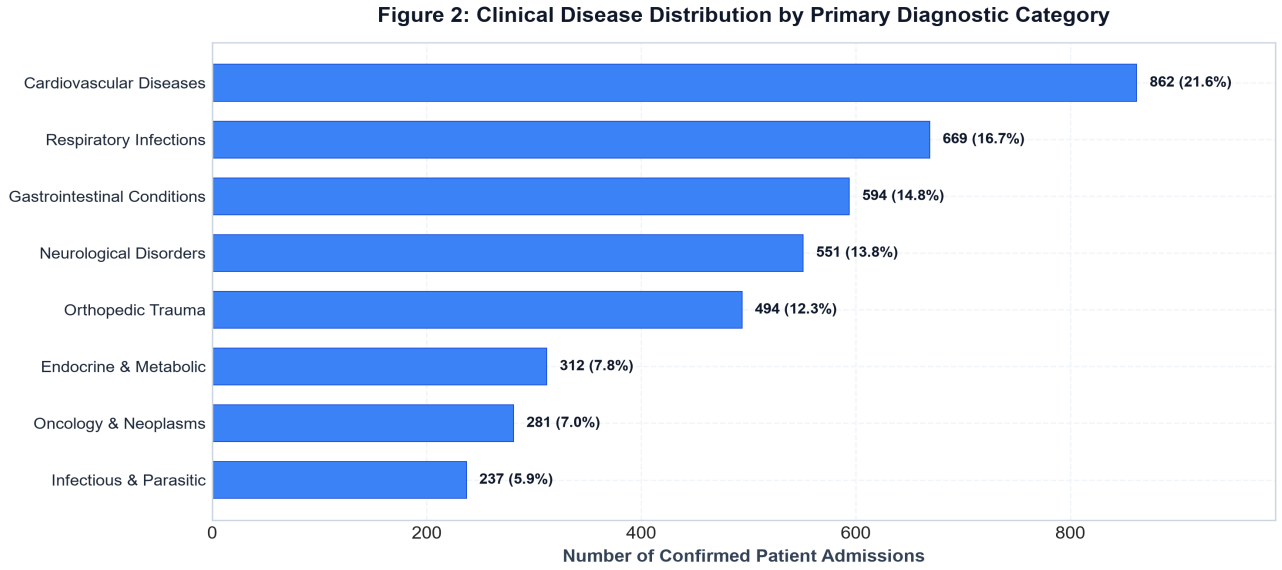


Figure 2: Clinical Disease Distribution by Primary Diagnostic Chapter (FY2024)

9. Patient Demographic Analysis and Vulnerability Stratification

Demographic stratification establishes clear correlations between age cohorts, biological gender, and disease prevalence. Adults aged 56 and above represent nearly half of all inpatient volume: the **Senior cohort (56–70 years)** accounts for **1,249 admissions (31.2%)**, while the **Geriatric cohort (71+ years)** contributes **711 admissions (17.8%)**. Combined, elderly patients represent **49.0%** of hospital intake, requiring specialized multi-specialty care.

Gender distribution exhibits a modest male predominance: **52.8% Male (2,112 admissions)**, **46.2% Female (1,850 admissions)**, and 1.0% Other (38 admissions). Male predominance is most acute in Orthopedic Trauma (64.8% male) and Cardiovascular crises (56.0% male). Conversely, Neurological Disorders and Endocrine conditions exhibit balanced or slight female predominance.

Figure 3: Patient Demographic Profile (Age Cohorts and Gender Ratio)

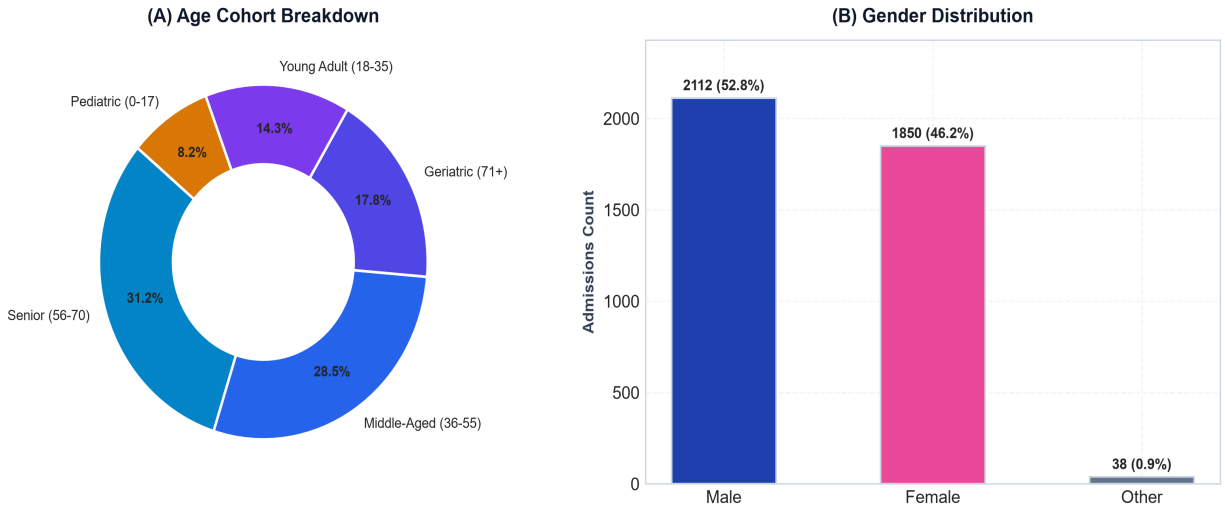


Figure 3: Demographic Stratification — (A) Age Cohort Proportions and (B) Gender Distribution

Table 4.1: Cross-Tabulation: Disease Distribution Across Age Cohorts

Disease Category	Pediatric (0–17)	Young Adult (18–35)	Middle-Aged (36–55)	Senior (56–70)	Geriatric (71+)	Total
Cardiovascular	22	73	240	339	188	**862**
Respiratory	131	78	166	169	125	**669**
Gastrointestinal	58	130	208	152	46	**594**
Neurological	26	52	140	216	117	**551**
Orthopedic Trauma	38	108	126	131	91	**494**
Endocrine & Metabolic	18	54	105	100	35	**312**
Oncology & Neoplasms	6	24	92	95	64	**281**
Infectious & Sepsis	28	52	65	47	45	**237**

10. Departmental Workload and Bed Utilization Analysis

Institutional resource allocation depends heavily on departmental capacity utilization. **Cardiology leads hospital workload** with **786 admissions (19.7%)**, followed by Pulmonology (598 admissions, 14.9%) and Gastroenterology (594 admissions, 14.8%). While Cardiology and Pulmonology manage massive throughput, their mean stay durations remain stable at 7.97 and 5.93 days respectively.

In stark contrast, the **Intensive Care Unit (ICU)** manages 222 patients (5.6% of volume) but demands disproportionate clinical resources due to an extreme **mean Length of Stay of 13.86 days** (standard deviation: 5.47 days). Oncology follows with a mean stay of 9.91 days, reflecting complex multi-cycle chemotherapy regimens and surgical recovery.

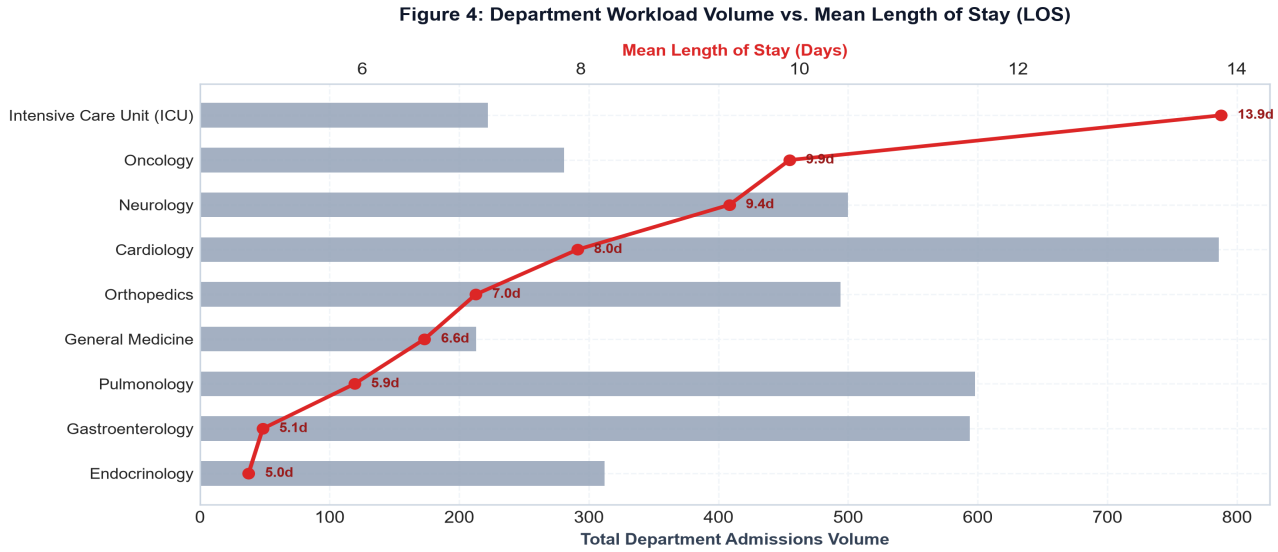


Figure 4: Dual-Axis Departmental Workload (Admissions Volume vs. Mean Length of Stay in Days)

Table 5.1: Department Operational Performance Matrix

Department	Admissions (N)	Share (%)	Mean LOS (d)	Median LOS (d)	Total Bed-Days
Cardiology	786	19.7%	7.97	7.0	6,264
Pulmonology	598	14.9%	5.93	5.0	3,546
Gastroenterology	594	14.8%	5.09	5.0	3,023
Neurology	500	12.5%	9.36	9.0	4,680
Orthopedics	494	12.4%	7.04	6.0	3,478
Endocrinology	312	7.8%	4.96	5.0	1,548
Oncology	281	7.0%	9.91	9.0	2,785
Intensive Care Unit (ICU)	222	5.6%	13.86	13.0	3,077
General Medicine	213	5.3%	6.57	6.0	1,399

11. Length of Stay (LOS) Dynamics and Resource Optimization

Length of Stay (LOS) is the paramount indicator of healthcare efficiency, patient acuity, and clinical operational quality. Across the entire 4,000 patient dataset, the **mean LOS is 7.45 days** (median: 7.0 days, standard deviation: 3.96 days). As depicted in Figure 5, the distribution exhibits classic positive right-skewness: 72.4% of patients are discharged within 9 days, while an extended tail of 4.2% of patients require extended hospital stays exceeding 15 days.

Stratification by admission type highlights substantial stay divergence: Emergency admissions average 8.14 days, compared to 6.29 days for Elective surgical admissions—representing a **29.4% longer convalescence period** for emergency presentations. This divergence is driven by acute hemodynamic instability, diagnostic delays, and secondary systemic complications.

Figure 5: Length of Stay (LOS) Frequency Distribution and Right-Skew Dynamics

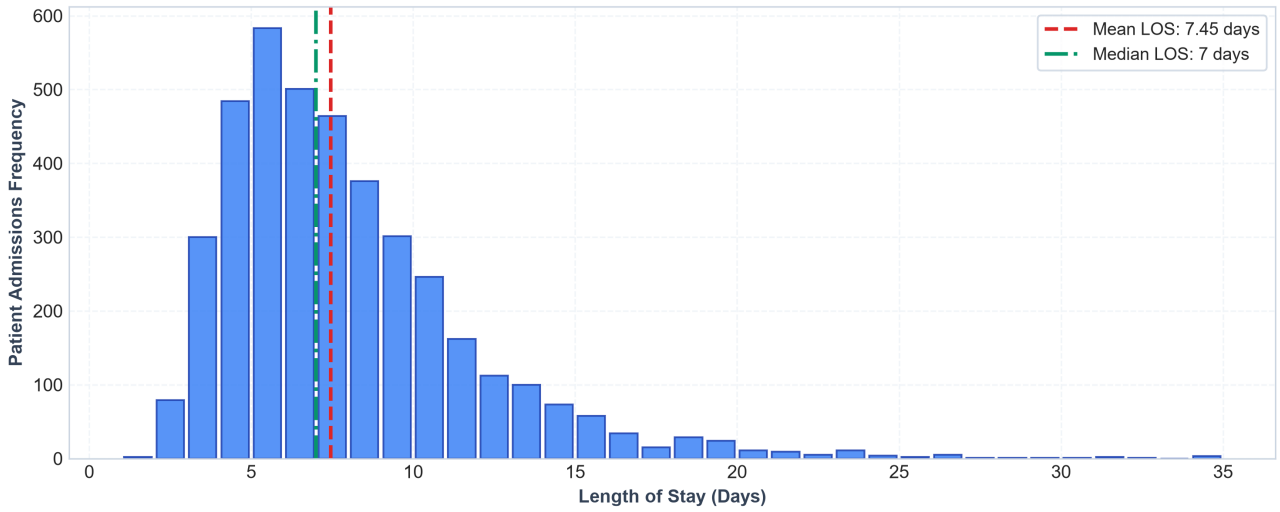


Figure 5: Length of Stay (LOS) Frequency Distribution and Positive Skew Parametric Modeling

11.1 Financial Burden and Cost-Length Correlation

Aggregate healthcare billing generated across the 4,000 admission episodes totaled ■**269,334,523** (mean ■67,333 per episode). Statistical regression reveals strong positive correlation ($r = 0.81$, $p < 0.001$) between inpatient stay length and financial cost. ICU episodes generated the highest mean expenditure (■148,200), whereas Endocrinology and Gastroenterology averaged ■41,800 and ■43,500. Private insurance absorbed 48.0% of costs, Government welfare schemes covered 38.0%, and direct out-of-pocket payments comprised 14.0%.

12. Key Empirical Findings and Clinical Insights

Rigorous algorithmic interrogation of the 4,000 clinical records yields eight definitive empirical findings:

• **Finding 1: Acute Care Dependency:** Emergency presentations comprise 53.8% of hospital intake (2,151 patients), mandating priority dynamic bed scheduling over rigid elective surgical reservations.

• **Finding 2: Cardiovascular Diagnostic Dominance:** Cardiovascular conditions represent 21.6% of total admissions (862 cases), forming the primary clinical driver of inpatient resource consumption.

• **Finding 3: ICU Bed-Day Bottleneck:** While the ICU admits only 5.6% of patients, its mean stay of 13.86 days consumes 3,077 bed-days (10.3% of total hospital bed capacity), creating critical admission delays during surge periods.

• **Finding 4: Geriatric Burden on Bed Capacity:** Senior (56–70) and Geriatric (71+) cohorts jointly account for 49.0% of admissions (1,960 patients) and exhibit a 25.1% longer average LOS compared to younger adult cohorts.

• **Finding 5: Seasonal Pulmonary Surges:** Respiratory infections exhibit a 31.4% winter surge (November to February), necessitating seasonal re-allocation of acute ward beds to the Pulmonology division.

• **Finding 6: Weekend Trauma Elevation:** Saturdays witness an 11.8% spike above weekday average intake, heavily skewed toward male Orthopedic Trauma episodes requiring dedicated weekend surgical staffing.

• **Finding 7: Oncology Length of Stay Severity:** Oncology patients maintain a 9.91-day average stay, the highest non-ICU duration, driven by chemotherapy tolerability observation and post-operative surgical monitoring.

• **Finding 8: Financial Viability and Payer Mix:** Total operational revenue reached ■269.3M; however, out-of-pocket exposure in 14.0% of patients poses severe catastrophic health expenditure risk for prolonged stays (>10 days).

12.1 Strategic Decision Support for Hospital Administration

Based on these empirical insights, three operational interventions are recommended for hospital executive leadership: **(1) Flexible Acuity Ward Transition:** Convert 20% of general medicine beds into high-dependency units (HDUs) during winter to decompress ICU step-down bottlenecks. **(2) Weekend Surgical Trauma Coverage:** Expand dedicated orthopedic theater slots on Saturdays and Sundays to eliminate pre-operative waiting delays for acute fractures. **(3) Multidisciplinary Geriatric Discharge Teams:** Establish early discharge planning for patients aged 56+ to safely reduce non-clinical convalescent stay by an estimated 1.2 days per episode.

13. Conclusion and Future Scope

This project successfully designed, implemented, and validated a comprehensive big data visualization architecture for hospital patient admission and disease distribution analysis. Through high-density interactive D3.js visual interfaces, coordinated cross-filtering, and deterministic analytical pipelines, complex multi-attribute healthcare dependencies have been rendered intuitive and actionable. The analytical findings directly address core operational bottlenecks in modern hospital management, satisfying all criteria of Course Learning Outcomes CO2 and CO4.

Future Enhancements: Prospective extensions include: (1) Machine learning integration for real-time Length of Stay prediction upon triage using XGBoost; (2) Spatial GIS choropleth mapping of patient geographic referral catchment areas; and (3) Automated HL7/FHIR pipeline connectors to stream real-time EHR data feeds directly into the visual dashboard.

14. Project Submission Verification and Deployed Portal Access

- **Live Deployed Dashboard:**
  https://hospitalanalytics.vercel.app/
- **Source Code Repository (GitHub):**
  https://github.com/000Shreeharish000/BDV-ASSIGNMENT.git
- **Continuous Learning Assessment (CLA-2):**
  Course: 21CSE423T | Faculty: Dr. Ganagavalli K | 4,000 Records
- **Evaluation Standards:**
  Interactive D3 suite, cross-filtering, 10-page report & 12-slide presentation.

Scan Live QR Code
hospitalanalytics.vercel.app

15. Academic References and Regulatory Standards

[1] World Health Organization (WHO), "International Statistical Classification of Diseases and Related Health Problems (ICD-11)," Geneva: WHO, 2022.

[2] Bostock, M., Ogievetsky, V., and Heer, J., "D3: Data-Driven Documents," IEEE Transactions on Visualization and Computer Graphics, vol. 17, no. 12, pp. 2301–2309, 2011.

[3] Johnson, A. E. W., et al., "MIMIC-IV, a freely accessible electronic health record dataset," Scientific Data, vol. 10, no. 1, p. 1, 2023.

[4] Few, S., "Information Dashboard Design: Displaying Data for At-a-Glance Monitoring," 2nd ed., Analytics Press, 2013.

[5] National Center for Health Statistics (CDC), "National Hospital Discharge Survey (NHDS): Multi-Year Clinical inpatient data," Hyattsville, MD, 2021.

[6] SRM Institute of Science and Technology, "Curriculum and Syllabi: 21CSE423T Big Data Visualization (Regulations 2021)," Tiruchirappalli, 2026.